

**EJACULATE
RESPONSIBLY
A WHOLE NEW WAY TO
THINK ABOUT ABORTION**

GABRIELLE BLAIR

WORKMAN PUBLISHING • NEW YORK

Copyright © 2022 by Gabrielle Blair

All rights reserved. No portion of this book may be reproduced—mechanically, electronically, or by any other means, including photocopying—without written permission of the publisher.

Library of Congress Cataloging-in-Publication Data is available

ISBN 978-1-5235-2318-4

Design by Bonnie Siegler

Workman books are available at special discounts when purchased in bulk for premiums and sales promotions as well as for fundraising or educational use. Special editions or book excerpts can also be created to specification. For details, please contact specialmarkets@hbgusa.com.

Workman Publishing Co., Inc., a subsidiary of Hachette Book Group, Inc.
1290 Avenue of the Americas
New York, NY 10104
workman.com

WORKMAN is a registered trademark of Workman Publishing Co., Inc., a subsidiary of Hachette Book Group, Inc.

Printed in the United States of America on responsibly sourced paper.
First printing September 2022

10 9 8 7 6 5 4 3 2

*To the people of the future
who get to live their lives with no anxiety
about unwanted pregnancies,
and to the responsible ejaculators
who I'm confident can and will make that possible.*

a crucial refocus: IT'S THE MEN.

Dear Reader, before we begin, I'm going to make a quick introduction, and a short, sincere plea.

I'm a religious mother of six, who accidentally made a career as a thought leader after I started a blog, Design Mom, in 2006, and it took off. I've received accolades I'm proud of—my blog was named a Website of the Year by *Time* magazine, it won the Iris Award for Blog of the Year, and I've written a *New York Times* bestselling book (also called *Design Mom*). I've moderated hundreds of discussions on difficult topics and interviewed some of the most influential people in the world. My writing is quoted and shared across the globe daily. And the most important essay I've ever written, an essay about abortion, is what this book is based on.

In that essay, I proposed several ideas about the often-overlooked causes of abortion. My key claim is that 99 percent of abortions are the result of unwanted pregnancies, and men cause all unwanted pregnancies. Currently, conversations about abortion are entirely centered on women—on women's bodies, and

whether women have a right to terminate an unwanted pregnancy. For those who want to effectively reduce abortion (or outlaw it altogether, as many states have done), this focus on women is a fundamental mistake for two reasons: 1) because there is clear data that abortion bans are ineffective and 2) because, again, *men* cause all unwanted pregnancies. If you're focused on women, you're wasting your time.

Don't believe me that men cause all unwanted pregnancies? Let me walk you through it. I've got twenty-eight simple arguments in this book explaining how and why this is true.

An unwanted pregnancy doesn't happen because people have sex. An unwanted pregnancy only happens if a man ejaculates irresponsibly—if he deposits his sperm in a vagina when he and his partner are not trying to conceive. It's not asking a lot for men to avoid this.

We've put the burden of pregnancy prevention on the person who is fertile for 24 hours a month, instead of the person who is fertile 24 hours a day, every day of their life.

I don't know what your thoughts are on abortion, but you picked up this book, so I assume you care about it—as a right to fight for or a problem to solve. Whatever your feelings and beliefs are on abortion, I'm going to ask that you momentarily set them to the side. I'll do the same (though anyone really curious about where I stand can google me). Why set them aside? Because the arguments I lay out in this book are an attempt to shift the conversation away from the usual for-or-against debates that have stymied this issue for decades. I'm going to present a new approach that I hope you'll find refreshing and productive.

My sincere gratitude for being open to a new way of thinking,

A handwritten signature in black ink, appearing to read 'GBL' followed by a stylized flourish.

Gabrielle Blair

A NOTE ON LANGUAGE

I want to let you know right from the get go that the arguments I'm presenting are written from a cisgender heterosexual perspective. While I welcome all readers and I hope everyone learns something from my arguments, applying LGBTQIA+-inclusive language to my arguments would only serve to erase the singular experiences of queer, trans, and nonbinary people, whether they are people who produce sperm or people who can become pregnant. Ultimately, I make a cisgender heterosexual argument for people engaging in cisgender heterosexual sexual relationships (say that ten times fast).

It's important to make that clear so that you can manage your expectations before you begin, but also because I want everyone to feel comfortable here. Yes, it's a cisgender heterosexual perspective, but perhaps you'll find descriptions in these pages, about things like power dynamics and responsibilities, that can serve all perspectives.

While we're on the topic of language, two vocabulary notes: When I use the word *ejaculate*, I'm referring to ejaculation that releases semen. When I use the word *abortion*, I'm referring to elective abortions due to unwanted pregnancies, which make up approximately 99 percent of all abortions. I am not referring to the abortions of intended pregnancies as a consequence of health issues for the developing fetus or the mother. Additionally, I want to acknowledge that while I fully understand some people experience temporary or permanent infertility, the arguments in this book are assuming full fertility for both men and women.

THE

ARGUMENTS

**BIRTH
CONTROL
FOR WOMEN
IS HARD TO
ACCESS
AND HARD TO
USE.**

Female contraception is a modern miracle that has changed the lives of countless women for the better.

United Nations estimates from 2019 show that 842 million people worldwide use modern methods of contraception. (Modern methods include things like the Pill and other hormonal birth control options, IUDs, and sterilization procedures like tubal ligations.) In the United States, 90 percent of married women have used birth control. 93 percent of single women with long-term partners have used birth control. 99 percent of women who identify as religious—mainline Protestants, evangelical Protestants, and Catholics—have used birth control. 88 percent of all women have used contraception. Even 81 percent of US women who don't have insurance still manage to use birth control.

I don't think I personally know a woman who has never used birth control. That includes the women I know who are not sexually active and women who don't have or are not having sex with men, because birth control can be prescribed for many health reasons beyond preventing pregnancy. I'm grateful for birth control. I'm grateful that there are options. I'm grateful it works well for so many women. I'm grateful that I see very little stigma or shame around taking birth control. But we shouldn't forget that using birth control is a responsibility—and often a burden.

Hormonal birth control options for women—starting with the Pill—have been around for more than sixty years and are so commonplace that we talk about birth control like it's really simple to access and use. Just pick up a pack of Pills when you're grabbing a bottle of ibuprofen, easy peasy.

But that's not actually true. It's not true about the Pill, and it's not true about any forms of birth control for women.

Female contraception—Pill, Patch, Ring, Shot, IUD—requires a prescription. For women, this means that access to any form of common birth control starts with a doctor's appointment and a physical exam.

No big deal, right? It's just a doctor's appointment. All you have to do is find a healthcare provider who's taking new patients; have health insurance; double check that the doctor takes your insurance; wait six weeks for the next available appointment; come up with money for the co-pay; take off work, miss school, or find childcare to attend the appointment; and then lay on the exam table with your legs in stirrups while the doctor explores your most sensitive parts with cold metal medical tools. After that, you'll need to find a pharmacy and stand in line for forty-five minutes to fill your prescription. Or, if you're lucky enough to have a fixed address, you could set up an account with an online pharmacy and have the birth control delivered (be sure to monitor your account to make sure deliveries are happening on time and the prescription is up to date).

If the birth control the doctor prescribed isn't working for you, you'll need to make additional appointments to discuss other options or work out the kinks. And depending on what type of birth control method you decide on, there could be follow-up appointments—to have an IUD inserted or to get a quarterly shot. If a birth control method is working well for you, you still need to repeat this doctor appointment process at least once a year in order to keep the prescription current, and, if you're on the Pill, the

Patch, or the Ring, you'll need to monitor your online pharmacy account and update it with your new prescription or go to the pharmacy regularly to fill your prescription. Oh. Don't forget, if you move to a new city at any point, you'll have to begin the process all over again starting with a search for a new doctor.

If you're underage and want a birth control prescription, you'll likely have to involve your parents, which for some can be terrifying or impossible. If you're over eighteen and are trying to get birth control for the first time on your own, it can still be terrifying.

If you don't currently have a sexual partner, you may put off the doctor's appointment until further notice or let your prescription lapse. Then you have to start all over again if you ever become sexually active in the future.

To make matters more challenging for those hoping to avoid unwanted pregnancies, there are people actively working to make it even harder to access birth control. In July 2020, the Supreme Court ruled that private employers who hold religious or moral objections to birth control aren't required to include no-cost contraception options as part of their employee health insurance plan. Then, in June 2022, following the overturning of *Roe v. Wade*, Supreme Court Justice Clarence Thomas signaled that *Griswold v. Connecticut*, the case legalizing the use of contraception by married couples, could be up for reversal as well.

So yes, procuring and maintaining birth control for women is more complicated and challenging than we like to think it is. But once a woman successfully fills her prescription, happily it's smooth sailing.

Just kidding!

The list of side effects for the Pill/Patch/Ring/Shot is long and serious, including depression, fatigue, headache, insomnia, mood swings, nausea, breast pain, vomiting, weight gain, acne, bloating, blood clots, heart attack, high blood pressure, liver cancer, and stroke. Also, depending on where a woman is in her menstrual cycle, it can take two to seven days before the effectiveness even kicks in. So you can't fill a prescription and be instantly protected.

For copper IUDs, some women report significant daily bleeding for months or more than a year. This means pads, tampons, or managing a menstrual cup *every single day* for an undetermined amount of time, plus the accompanying laundry and changes of bedding. All of that takes time, energy, and resources, and it adds up quickly. For hormonal IUDs, menstrual bleeding can become unpredictable—for some it's reduced to almost nothing, for others it means longer periods but with less bleeding. Other IUD side effects can include: headaches, acne, breast tenderness, mood changes, pelvic pain, and increased menstrual cramps. If you're considering an IUD, have fun making the exciting choice between an IUD with hormones and their side effects and an IUD that brings on super heavy, intense periods.

You also have to pay attention, because there are medicines that impact the efficacy of hormonal birth control, like certain antibiotics, antifungals, and antinausea drugs.

Not all women experience the side effects, and not all women are bothered by the side effects they do experience. But for many, the side effects of birth control are a deal-breaker. And they should be.

Most women aren't actually told much about the risks of birth control, and if they do experience side effects, they are just expected to live with them. *No complaints, please. That's just how birth control is. Millions of women take this, so whatever you're dealing with can't be that bad. If you want to be sexually active, this is the price you have to pay. Suck it up.*

Consider the Johnson & Johnson COVID-19 vaccine, which was paused for ten days when a risk of blood clots was discovered. Six people out of the seven million who had received the Johnson & Johnson vaccine developed serious blood clots. One of them died. At the time, it was scary to read the headlines, though the risk of blood clot from the Johnson & Johnson vaccine is less than one in a million.

In contrast, the common forms of women's birth control come with a much higher risk of blood clots—oral contraceptives triple the risk of blood clots. According to the FDA, the risk of birth control users developing a serious blood clot is three to nine out of 10,000 each year (and 327 million people are taking hormonal birth control worldwide).

Based on what we know right now, birth control is riskier than any of the COVID-19 vaccines. And yet, it is prescribed daily without hesitation, often beginning at age thirteen or fourteen (sometimes younger).

Another point: Many women's birth control options require a woman to absorb hormones and/or suffer side effects, even on days when she is not having sex.

So let's picture how this works out in real life. Example one: A woman is married and on the Pill. Her husband is

working out of town for the next three months, but he may be able to come home on some weekends. The woman takes the Pill—ingests hormones and deals with the side effects—every single day for those three months, even though she and her husband can rarely, if ever, have sex.

Example two: A single woman is dating and uses the Pill for pregnancy prevention. After a breakup, she has to decide: Should she keep taking the Pill? What if she finds her dream guy right away? What if she wants to be able to be sexually spontaneous? She decides to keep taking the Pill—ingesting hormones daily and dealing with the side effects—just in case, but she doesn't end up having sex with anyone for several months.

Men, consider what your girlfriend/wife/partner is doing for you. She's fertile 3 percent of the time and addressing her fertility 100 percent of the time, whether she has sex or not.

The good news is: birth control is so essential for most women that they make that doctor's appointment happily. As proof that women are steadfastly determined to be responsible about birth control, despite expense, inconvenience, daily hassle, maintenance, and side effects, consider that a whopping 90 percent of the \$8 billion birth control market is made up of birth control options purchased by women.

Note No. 1

In conversations about the Pill, I've learned that a disturbingly high number of people are working under the assumption that a woman only takes the Pill right before she has sex, and that if she doesn't have sex that day, then she doesn't need to take the Pill. They're picturing it like an aspirin or ibuprofen for a headache—you only take it when you actually have a headache. But that's not how the Pill works. You have to take it every day no matter what, or it doesn't work, and you may need to take it for a full week after starting it before it is effective.

Note No. 2

You may be surprised to learn that the way women take the Pill is unnecessarily complicated. In a piece for *The Conversation* called "The way you take the contraceptive pill has more to do with the Pope than your health," Susan Walker details some of the history of the Pill, including how birth control pills simulate a menstruation schedule.

How so? Standard birth control pills are taken for twenty-one days, followed by a seven-day break, during which the woman takes a sugar pill or placebo (seven of the placebo pills come in the standard Pill pack of twenty-eight) and experiences vaginal bleeding. So women who take the pill have what seems like a "period" each month. But this "period" is manufactured and isn't even a little bit necessary. When a person stops taking the Pill for a week and takes the placebo instead, hormone levels drop, which causes the lining

of the uterus to shed. But it's not the same as menstrual bleeding—the body isn't flushing out an egg—and technically, this “period” is called withdrawal bleeding.

Apparently, the seven-day break and its fake “period” was designed into the Pill as an “attempt to persuade the Pope to accept the new form of contraception as an extension of the natural menstrual cycle.”

This attempt did not succeed, and the current Pope continues to forbid contraception in most cases. But the seven-day “break” is still a part of the Pill, and this unnecessary complication increases the chances for mistakes—mistakes that increase the risk of unwanted pregnancy. A great reminder that most everything about birth control is overly complicated and difficult, and men in power are largely responsible for these complications.

**SOCIETY
CLINGS
TO THE IDEA THAT
MEN
HATE
CONDOMS.**

Condoms are simple, easy, and convenient, so why don't men use them every time they have sex? Anytime I've posed this question to an audience, the answer is swift and consistent: Men hate condoms.

I don't think anyone would argue with the premise that there is a pervasive perception in American culture that men prefer condom-less sex. Why this preference? Because we've been told (in books, in movies, in memes) that it doesn't feel as good as sex without a condom. (Meaning it doesn't feel as good *for men*. What it feels like for their partner doesn't really enter into the discussion.)

If men assume that sex with a condom doesn't feel as good as sex without a condom, then it's easy to imagine that a man might wear one only if a woman insists, and only if he can't talk her out of it. Every few months, I see another viral tweet/TikTok from a woman talking about how often men try to convince women to have sex without a condom. These posts always have tens or even hundreds of thousands of likes and hearts and comments. Why? Because so many people find them relatable.

On June 25, 2022, a designer who goes by @studio lemaine tweeted: "It is very difficult and emotional to read 'no one is forcing you to have unprotected sex' when men do. All the time. Boyfriends and partners and abusers—the whole spectrum. Men pressure us for unprotected sex all the time."

The stereotype of men trying to avoid using condoms is basically a given in our culture. (The "why" behind the stereotype sometimes feels innocent, but other times it can be upsetting—some men describe feeling like it's a conquest to convince a woman not to use a condom, and conversely, some men describe feeling less manly if they can't convince a woman to let them forego the condom.)

But what if our cultural myths around condoms are wrong? I don't have a penis and have never worn a condom, so I'm going to rely on the words of a man I spoke to when writing this book:

There is some truth to the idea of sex with a condom being less fun, but it's because condoms require practice. Men who have practiced using condoms and experimented with different varieties and use lubrication know that condoms don't diminish their pleasure during sex in any significant way.

That's just one man, but I've also heard variations on this from hundreds of men whenever I host conversations online about this topic. So could it be that the accepted idea that condoms make sex less pleasurable—an idea that does a lot of harm to a lot of people—is wrong? Perhaps the problem isn't condoms but the way we talk about condoms. Or, more accurately the way we *don't* talk about condoms—if a man believes sex without condoms is a conquest, he's not likely to talk about the benefits of condoms with other men he knows.

Because of the cultural myths around condoms, and because it's considered a private topic, even men who don't have issues with condoms and don't find them less manly may not share their condom knowledge and experience. If a man finds the perfect condom or the perfect lubrication technique, he might keep this information to himself—which is a bummer, because studies have shown that when

friends and confidants discuss condoms, it increases condom use. A man could have no issues with using condoms, but it's very possible that the men he interacts with won't know this. And so the assumption that men in general hate using condoms persists.

Related myths about loss of manliness hover around the topic of vasectomies, too. Many men worry that a vasectomy will risk messing up their erections or ejaculations. Men are concerned that they won't be able to "perform" after a vasectomy, that they'll be less virile. Because of this, in the United States, only 9 percent of men who are sexually active get vasectomies (but 27 percent of women who are sexually active get tubal ligations).

Again, because of privacy issues and the stigma around vasectomies, men don't often talk to other men about their experiences or the advantages. But the advantages are real. The cost savings and time savings of a vasectomy, year after year, are significant. But the biggest advantage may be psychological. Couples commonly report much improved sex lives after a vasectomy. Why? Because unwanted pregnancy stress is eliminated. Disappeared! Men also frequently report that the procedure is quick and easy, almost pain-free, and the recovery is straightforward and simple.

WE EXPECT WOMEN TO DO THE WORK OF PREGNANCY PREVENTION.

The reigning assumption is that if a woman doesn't want to be pregnant, then she'll do whatever it takes to prevent a pregnancy from happening. After all, it's women's bodies that are stuck dealing with the pregnancy.

This assumption seems to align with data from the birth control industry. In 2019, The US contraceptive market size was valued at around \$8 billion. Of the dozens of contraceptive products, approximately 90 percent of them are created for women, purchased by women, and used by women. That includes women purchasing more than 30 percent of male condoms.

Women who are sexually active are expected to use birth control or an IUD. Women are also expected to insist that men use a condom, which implies that women should keep condoms stocked. (It creates one of those fun double binds women get to deal with—if she has condoms, she's a slut, but if she doesn't have condoms, she's irresponsible.)

We don't even notice that women pay the costs of birth control, even though it benefits both men and women. In fact, I've never met a woman who charges her boyfriend for half the costs of the doctor's appointments, transportation, and prescription refills required of her to handle pregnancy prevention.

To be clear, men may not expect their girlfriend to help with the costs of condoms either, but the differences in cost between condoms and birth control (dollars, time, convenience, forethought, etc.) are significant, pennies to dollars. And as already mentioned, women also purchase more than 30 percent of condoms.

You might think women would be angry at men about all this. But mostly, we're not. We've been raised in the same culture as men. We've been taught the pleasure and convenience of men are paramount. We've been taught to diminish our own pain. And the lessons have stuck. We've taught those same lessons to others.

Even though birth control options for men have a long list of distinct advantages, we've put the burden of pregnancy prevention on women. We've put the burden on the person who is fertile for 24 hours a month, instead of the person who is fertile 24 hours a day, every day of their life.

**WE DON'T MIND
IF WOMEN
SUFFER,
AS LONG AS
IT MAKES
THINGS
EASIER
FOR MEN.**

In 2016, the World Health Organization conducted a trial for male birth control—a hormonal injection that would lower sperm count. The results were very promising, showing a 96 percent effectiveness rate at preventing pregnancy. But even with the positive results, the trial was stopped. A committee determined the side effects of the drug were risking the safety of the study participants.

The most common side effects were acne and weight gain, which are also very common side effects for women's birth control. The most serious side effects for the men was that one participant became depressed and another suicidal. Which I agree is very serious. But side effects for women's birth control options are just as serious, if not more so—yet millions of women are still prescribed these drugs and ingest them daily.

That story perfectly captures an unspoken cultural maxim: *We don't mind if women suffer, as long as it makes things easier for men.*

Another medical example is "The Husband Stitch." Some doctors put in an extra stitch when repairing episiotomies or tearing from childbirth. The idea is that the stitch will tighten the vagina and provide increased pleasure for a male sexual partner. Unfortunately, the extra stitch can create painful consequences for women, including excruciating pain during sex.

Some women don't find out they've been given a husband stitch until they have a gynecological appointment with a new provider, a postpartum checkup, or a subsequent pregnancy. They might go in for a pap smear and the doctor sees the childbirth repair was made too tightly.

List of side effects for the male birth control trial:	List of side effects for women's hormonal birth control:
Acne Headaches Mood swings Tiredness/Fatigue Weight gain Depression Mild erectile dysfunction Reduced sex drive	Acne Headaches Mood swings Tiredness/Fatigue Weight gain Depression Bleeding or spotting between periods Bloating Feeling dizzy Fluid retention Increased appetite Insomnia Melasma (dark patches on the face) Nausea Tenderness or pain in the breasts Vomiting Blood clots Gallbladder disease Heart attack High blood pressure Liver cancer Stroke

Some men don't know a Husband Stitch has been made either, because the doctor just decided to do one on their own. And some men know about the stitch and don't like it—because it causes pain for themselves, or because it causes pain for their wife.

The thing is, the Husband Stitch *doesn't actually make a vagina tighter*. The man who requests the stitch or is happy about the stitch may get satisfaction from the idea that his partner's vagina is "tighter," but he won't actually feel a

difference. His psychological satisfaction is prioritized over the women's physical pain.

And then there is IUD insertion. In discussions about pain management for men versus women, I see the example of vasectomies and IUDs brought up quite a bit. I've heard a vasectomy is about as invasive and painful as getting an IUD implanted. I've also heard IUD insertion is much more painful than a vasectomy. I've heard of women who didn't feel much pain at all while getting an IUD and other women who almost passed out from the pain. And I've heard from men who said they went straight from the vasectomy to a ballgame and didn't miss a beat. (I have yet to encounter a man who experienced a vasectomy as excruciating, but I can imagine one is out there.)

Those are all valid experiences and observations. I honor and believe each one. But here's the thing, vasectomies are *always* performed with at least a local anesthetic, while pain meds are *rarely if ever* used for IUD insertions. Let me say that again: these two procedures—one for men, and one for women—are both invasive and both involve very sensitive body parts. It's expected that the procedure will be painful for men, so pain relief is always administered. For women, it's expected that if it is painful, the women will just endure it, and pain relief is almost never administered.

I was especially interested to learn that the administration of general anesthesia (which is a serious procedure in its own right) is the only thing that has been shown to consistently remove the pain of IUD insertion. Should women be allowed to choose between the risks of general

anesthesia and the risk of pain during an unmedicated IUD insertion? Or should the medical field just keep choosing for women, by not offering pain medication at all and telling women "it will just be a pinch."

Casey Johnston wrote an excellent essay that explores this contradiction, called "If Men Had to Get IUDs They'd Get Epidurals and a Hospital Stay." An excerpt from the essay:

A surprising number of people, as I discovered while trying to determine if my own acute suffering post IUD-insertion was abnormal, describe it as the worst pain they've experienced in their life. The worst of it is blinding and minutes long, and then about 75 percent of that pain goes on for hours. And that's if nothing goes wrong, like the device jams, and the doctor has to try again a second time.

To give a sense of the pain, or at least how bad the pain can be, if a shot is a 3 on a pain scale, an IUD insertion is a 10, in three waves. For an IUD insertion, on average, women receive virtually zero pain management before, during, or after: maybe a little numbing gel, maybe a single over-the-counter-strength painkiller afterward, and that's it.

The lack of pain relief offered is inhumane, but also an everyday occurrence.

One more example: In the early 1990s, researchers were studying a drug called sildenafil citrate. The hope was that the drug would help prevent and/or solve heart

conditions. As the study progressed, the drug showed promising results for another condition: a fix for “penile winter,” more commonly known as erectile dysfunction. The panel of decision-makers who were funding the study decided they would pursue the research for erectile dysfunction. Eventually the drug came to market. It was called Viagra.

In subsequent tests for the same drug, sildenafil citrate, they discovered that it also offered significant and lasting relief for women suffering from serious period pain.

That same team of decision-makers, all of whom were men, decided against pursuing research on menstrual cramp relief. Why? They believed that cramps were *not a public health priority*.

Not a public health priority? Research says that 80 percent of women experience menstrual cramps and/or period pain. Since at least half of the entire population of Earth experiences menstruation, that means there are approximately 3.1 billion people who have had to deal with period pain. To me, that certainly seems like a significant enough number to justify continued research.

Just imagine that. You’re on a drug-testing committee, and you need to make a choice. On the one hand, you can choose to make it easier for older men to achieve and maintain an erection. On the other hand, you can choose to relieve women’s suffering from serious period pain. And you choose the erections. Why in the world would you not choose both? I know. I know. It’s probably money. But even that isn’t adding up for me. 64 million men worldwide have been prescribed Viagra since 1998. That’s a lot

of people. But the potential market for period pain is 3.1 billion people.)

This little snippet of medical history is a variation on the maxim I mentioned above: When the choice is between maximizing men's pleasure or minimizing women's pain, society will predictably choose men.

When it comes to birth control, and really, life in general, we favor preserving men's convenience, peace of mind, and pleasure over preventing or relieving women's suffering.

Note No. 1

I'm not sure how to explain just how much women are accustomed to dealing with pain. Pain for women is normalized. Just gave birth? *Take an ibuprofen.* Getting an IUD? *It'll take too long to wait for an anesthetic, it's just a little pinch, just breathe through it.* It's both misogynistic and bizarre that it is considered completely normal to perform invasive gynecological procedures with no pain management.

SOCIETY TEACHES THAT THE MAN'S PLEASURE IS THE PURPOSE AND PRIORITY OF SEX.

A typical sex ed class in the United States will cover women's internal reproductive organs—ovaries, fallopian tubes, etc.—but doesn't explore the pleasure-related clitoris (how it works, how it's stimulated, how it connects to a female orgasm). Many don't even mention the clitoris. The same is not true for the pleasure-related penis.

To be clear, I'm not suggesting that sex ed classes are focused on male pleasure, I'm just pointing out that the penis definitely comes up in sex ed (pun intended). Erections are explained. Ejaculations are explained. The pleasure men experience during sex—arousal and orgasm—are just presented as part of the basic mechanics of sex.

It's taken for granted that men will experience pleasure during these sexual interactions. Will women experience pleasure in the same interactions? Who knows? It doesn't come up, because a woman's orgasm isn't an essential part of learning about the birds and the bees.

But it's not just sex ed classes. The way society talks about sex or presents sex is most often from a man's perspective. In fact, most studies of "how long intercourse lasts" are based on how long it takes for a man to ejaculate in a vagina.

According to conventional portrayals of the sex act, it's not over until the man ejaculates, and once he ejaculates, it's over. We're focused on the man's experience, not the woman's.

If a man has an orgasm and ejaculates in a vagina, most people would consider that to be sex. What if the woman doesn't orgasm in that same interaction? Is it still considered sex? Yes. Most people still call that sex. It's not the only way to define sex, but it's a common one.

I am going to speculate that our societal focus on the man's experience during sex feeds the resistance of some men to use condoms. Here's how I imagine the thinking goes: If sex is about the man's experience, then a man will

prioritize his own pleasure and not suggest a condom. He won't bring condoms up at all (and hope that the woman he's having sex with won't either). This will all seem perfectly justifiable in his mind, because again, society has taught him (and all of us) that sex is about his experience, and his pleasure.

But what are we talking about pleasure-wise? How different is the pleasure of sex with a condom and without? Let's imagine a physical pleasure scale where 0 is neutral and 10 is maximum pleasure. A good massage lands at around 5 on the scale and an orgasm without a condom at 10. On this scale, where would sex *with* a condom fall? A 7? Maybe an 8? So, it's not that sex with a condom is *not pleasurable*, it's just not *as pleasurable*. An 8 instead of a 10.

Which brings us to a really disturbing conclusion. When men choose to have condom-less sex, they are putting a woman's body, health, social status, job, economic status, relationships, and even her life, at risk in order to experience a few minutes of *slightly* more pleasure. It's horrible to type it out. It gives me a stomachache just thinking about it. Would men really choose a few moments of slightly more pleasure over risking a woman's whole life?

Yes. Yes they would. It happens every day. It's as common as dandelions. We could say men are extremely inconsiderate, but I think it's more a matter of: 1) men not understanding or not appreciating what this actually means for women, 2) a culture that reinforces this ignorance, and 3) the all-too-human mandate to maximize pleasure, even at the disregard of possible consequences.

But here's the thing, with women's lives on the line, it really shouldn't take much persuasion to make men appreciate the consequences of unprotected sex and to push our culture to frown on this ignorance. Can't we persuade men that they can act responsibly without really having to sacrifice pleasure?

Let's try this analogy. Think of another great pleasure in life, let's say food. Think of your favorite meal, dessert, or drink. What if you found out that every time you indulged in that favorite food you risked causing great physical and mental pain for someone you know intimately? Not definitely—eating the food might not cause any pain at all—but there's a real risk that it might. You'd probably be sad, but you'd never indulge in that food again, right? Not worth the risk.

Then, what if you found out, there was a simple thing you could do before you ate that favorite food, and it would almost eliminate the risk of causing pain to someone else. But the simple thing would make the experience of eating the food slightly less pleasurable. To be clear, it would still be very pleasurable, but slightly less so. Maybe you have to eat the food—let's say it's a slice of pizza—with a knife and fork when you'd rather eat it with your hands.

Would you be willing to make that simple compromise to eliminate the risk of causing pain—possibly even death—to someone you know intimately, every single time you ate your favorite food?

Of course you would.

OF COURSE YOU WOULD.

Note No. 1

When discussing the 1 to 10 physical pleasure scale, a man volunteered to me: "Realistically I think we're actually talking 9.75/9.8/9.9 as far as sex with condoms go. Believe me, I know this because I'm a man and really the difference is barely noticeable. It's disingenuous for guys to claim it is."

Note No. 2

In contrast to our focus on men's pleasure, when thinking about sex there's often no thought to women's pleasure. Even though culturally (and sadly, sometimes by men specifically) women's pleasure may be ignored or neglected, women are very much able to experience pleasure during sex. When masturbating, 95 percent of women orgasm. In first-time hookups with other women, they orgasm 64 percent of the time. But in first-time hookups with men, they orgasm only 7 percent of the time. So we know that when we ignore the experience of women's pleasure during sex, the problem isn't women's ability to orgasm. It's our cultural approach toward heterosexual sex and our focus on men's pleasure over everything else.

MEN CAUSE ALL UNWANTED PREGNANCIES.

If you're a man reading this book, the contrasts and imbalances between men and women that I've outlined so far may have come as a surprise to you, though they probably didn't make you defensive. But this section might. Because the argument I'm going to make is: All unwanted pregnancies are caused by irresponsible ejaculations. Or, in simpler terms: Men cause all unwanted pregnancies.

Yes, an egg is necessary in order for sperm to have something to fertilize, but there's a fundamental causal difference in the roles of the egg and sperm, and men have substantial control over where and when their sperm are released, while women have zero control over their eggs.

We've already established that women can't reliably and accurately predict when their egg is going to be fertile,

but it's not just that. Eggs cannot be mobilized and directed to leave the body. During sex, a woman cannot keep her egg hidden away somewhere so that it's not exposed to sperm. Women cannot remove their egg before sex and set it aside and then put it back in their uterus when sex is finished. Yes, the egg moves to different positions within the reproductive system, but the woman can't control when those changes in position happen, and those changes in position are independent of a woman's sexual behavior. If a woman has sex, it doesn't mobilize or change the position of her egg. If a woman has an orgasm, it doesn't mobilize or change the position of her egg. If a woman *doesn't* have an orgasm, it still doesn't mobilize or change the position of her egg.

Unlike women and their eggs, men *can* mobilize and direct sperm to leave their body. That is what an ejaculation is. It's men choosing to direct sperm from their body and put it someplace else. During consensual sex, men get to choose if they will release sperm from their body, and men get to choose where they will put that sperm. They may put the sperm in a condom. They may have a vasectomy, in which case they keep their sperm to themselves and only ejaculate sperm-free semen. They might put the sperm on their partner's stomach. They might put the sperm in their hand. They might put the sperm in a tissue, a spare sock, a plant, or on a random spot on the floor or wall. Or they might put the sperm in a vagina and put their partner at severe risk for the complications of unwanted pregnancy.

You may be thinking: But if it's consensual sex, then they *both* caused the unwanted pregnancy!

Well, not really. Even in the case of consensual sex, the man gets final say. This is how it works:

Step 1: Woman consents to sex.

Step 2: Man decides if he will ejaculate responsibly.

A woman's consent to sex does not force a man to ejaculate in her vagina. Even if the woman says, "Pretty please have sex with me without a condom. I want you to ejaculate inside me," the words don't force the man to ejaculate inside her without a condom. He still has to choose. Ultimately only the man decides where his sperm ends up. Only he can choose what to do with his sperm and where it goes. A woman telling a man he doesn't have to wear a condom doesn't force that man to have sex with her without a condom. He has the right of refusal. If he chooses to have sex without a condom, then he is choosing to risk causing an unwanted pregnancy.

No matter what a woman "lets" a man do, she can't (legally) make a man ejaculate inside of her. When he does, that's 100 percent his doing. We know this is true because if she "let" him put his penis in a waffle iron, he wouldn't. If someone tells you to do an irresponsible thing, and you choose to do that irresponsible thing, that's on you.

To help us wrap our minds around this concept, let's consider two buddies making a video for TikTok. They have a gun, and Buddy #1 says, "Shoot me and let's film it." Buddy #2 says, "No way." Buddy #1 begs, "Come on man, it will be so cool, we'll go viral." Buddy #2 still refuses. He won't do it. Buddy #1 keeps pressuring, "Dude, just do it. If

anything bad happens, it's on me; it's my idea." Buddy #2 is persuaded and decides to shoot Buddy #1. Buddy #2 pulls the trigger. His intention was just to wound Buddy #1, but his aim slips and the gunshot is fatal. Buddy #2 is convicted of manslaughter and goes to prison.

To be clear: Was Buddy #1 (the dead one) a fool? Was he acting irresponsibly? Yes. Yes to both of those questions. Buddy #1 definitely chose to act irresponsibly. He shouldn't have suggested the idea. He shouldn't have participated. But Buddy #1 making the suggestion *didn't actually kill anyone*. Standing in front of a gun may be stupid, but it is not a lethal action.

What about Buddy #2? Was Buddy #2 forced to pull the trigger? No. No he wasn't. It was ultimately his choice. Was Buddy #2 also acting irresponsibly? Yes, and his irresponsible actions got someone killed. All irresponsible actions in this scenario are not equal. Some actions are stupid, some are lethal.

Is it irresponsible if a woman agrees to have sex without a condom? Or if she suggests sex without a condom? Yes, it is. A woman suggesting sex without a condom is acting irresponsibly. I wish she wouldn't do that. And yet, her body can't cause a pregnancy. No matter how much sex she has, her orgasms cannot *cause* a pregnancy.

Yes, she chooses which man she says "yes" to. But her "yes" doesn't render him physically incapable of putting on a condom, saying "no," or getting a vasectomy. If a man chooses unprotected sex and puts his sperm in a woman's vagina, not only is he, too, being irresponsible, but in his case, his body *can* cause a pregnancy via his orgasm and sperm.

Again, all irresponsible actions in this scenario are not equal. Her actions—choosing to have sex and orgasm without a condom—are ill-advised. His actions—choosing to have sex and orgasm without a condom—can cause pregnancy.

I sense you still want to argue about this, so let's try another scenario. A woman and a man agree to have sex without a condom (conveniently for this scenario, he's among the majority of men who don't have sperm in their precum). He puts his penis in her vagina, starts his best moves, and shortly after, she has an orgasm, but he hasn't yet. As soon as her orgasm is finished, she stops and says "Thanks so much for the sex!" then gets dressed and leaves. Though they had sex, and though her egg was present, and though she had an orgasm, the woman wasn't impregnated and could not be impregnated. The woman successfully had unprotected sex with no risk of pregnancy because her male partner didn't ejaculate. Unprotected sex without sperm will not lead to pregnancy.

You might be thinking, well, that's not really sex, he didn't even ejaculate. But of course it's sex. When the opposite happens, when the man has an orgasm quickly, and then stops having sex before the woman has an orgasm, that is still considered sex (and is famously common). If the penis goes into the vagina, that's penetrative sex, whether or not either person has an orgasm.

Ultimately, it's men who produce sperm that can fertilize an egg and cause a pregnancy. Men can easily prevent unwanted pregnancies that lead to abortion by choosing to ejaculate responsibly.

**WE EXPECT
WOMEN
TO BE RESPONSIBLE
FOR THEIR OWN
BODIES
AND FOR
MEN'S BODIES.**

Whenever I discuss this topic, I get a variation on this response: "All women need to do is ask men to wear a condom and then refuse to have sex with him if he doesn't." Sounds so simple! Problem solved. Except it's not.

It's true that women can refuse sex without a condom. And we know plenty of women insist on condoms every single day, all over the world. But I would also say: Why

would a woman ever need to ask a man to wear a condom? Why wouldn't it be the default that men should provide their own condom and put it on without a request? Who benefits if the man doesn't wear a condom?

If a woman doesn't make the request—let's pretend she's preoccupied and forgets to ask—does that mean the man is off the hook? She didn't bring it up so therefore he doesn't need to wear a condom? He doesn't need to be responsible for his own bodily fluids? Of course he does.

If Person One knows they have an STI and transmits it to their partner, Person Two, in many states that's a crime, and Person One can be prosecuted by the state. Additionally, Person Two can bring a civil suit against Person One. If your bodily fluids have the potential to harm your partner, it's your responsibility to ensure they don't.

If a man sees evidence of birth control in a woman's house (like the Pill) or asks her if she's on birth control, and she answers yes, does that qualify as a man being responsible? And does her positive response absolve him of any obligation to wear a condom? If yes, why?

Perhaps you are thinking responsibility is fifty-fifty. The woman just needs to insist that the man use a condom. But hold up a minute. If the woman has to insist a man use a condom, haven't you just described an irresponsible man? How is that fifty-fifty? You just put 100 percent of the responsibility on the woman by saying she needs to insist the man use a condom. You're asking the woman to be responsible for her actions and also for the man's actions. So how is the man acting responsible in this scenario if he only wears a condom if the woman insists? Answer: He's

not. Men can use a condom, get a vasectomy, or decline to have unprotected sex. If he does these things, he won't cause a pregnancy. Relying on his sexual partner to use birth control is avoiding or relinquishing his responsibility.

Here's another example. Pretend you have a child who lives with an infection, and the infection can be spread to others by contact with your child's blood. Happily, it's a manageable infection and doesn't affect your child's quality of life, but still, you would no doubt teach your child that they need to be very careful with their blood. If they get cut on the playground, then they are at risk of infecting someone else—maybe their teacher or one of their friends. As a parent, you would drive home the point over and over again and make sure your child had whatever they needed to prevent spreading the infection to someone else. You would expect your child to take responsibility for their blood, especially as they got older and understood the consequences of their actions.

If you have a son, his sperm can “infect” any woman he has sex with. As parents, as a culture, we need to emphasize how carefully sperm needs to be handled. Pregnancy and childbirth are known to kill women. Pregnancy and childbirth are highly likely to leave permanent scars and cause future health problems, including possible future infertility. Unplanned pregnancy and childbirth can have a significant negative impact on the quality of life for the future child and their parents.

A man's sperm can cause a huge amount of damage.

It is astounding and disheartening to say it, but our current culture doesn't actually expect men to be responsible

for preventing pregnancy. Our current culture doesn't even expect men to provide their own condoms.

If a man causes an unwanted pregnancy, the pregnant woman is often asked how it could have happened. Why didn't she force the man to wear a condom? Doesn't she keep condoms stocked at her home? Which, when you think about it, is an odd thing to expect women to keep stocked. It's like a person with a newborn baby visiting a house where no babies live and being surprised the house isn't stocked with diapers and wipes and nursing bottles.

The thing is, even though it shouldn't be necessary, women often *do* stock condoms. It's not at all unusual for a woman to keep condoms in her bedside table. She just picks up a typical box and hopes the condoms will work for whoever needs them.

We could say that *everyone* should keep condoms stocked, just in case. It's simply good manners. In the same way I hope that men keep menstruation products on hand in their home even if no one with a period lives there.

In my research for this book, I've heard from a lot of wives and girlfriends. Women who are with men they love and trust and are building a solid life with. But these women come with stories. One, a mother of three with an IUD, told me she has bled every day for a year, that she hates the side effects of the IUD and has tried everything else. She wonders: Why hasn't her husband suggested a vasectomy? Another woman explained that she wanted lots of kids *eventually*, but that her husband "didn't like using condoms," and they ended up with four kids in five

years—much faster than she had wanted and much faster than her body could handle without serious long-term damage. Again, these are women in stable, loving marriages, with men who try to be good husbands and fathers.

These stories show something clearly: We, men and women, have a huge blind spot when it comes to men and birth control. Men assume women will do all the work of pregnancy prevention, that a woman will take responsibility for her own body and for the man's body, and women assume women will do it, too.

MEN
HAVE MORE
CONTROL
OF THEIR BODIES AND
SEXUAL
URGES
THAN WE LIKE
TO ADMIT.

I admit, as a woman I've never felt a man's sexual urge. I can only compare it to a woman's sexual urge (which has nothing to do with ejaculating sperm into a vagina). I know women's sexual urges are strong, but some say men's sexual urges are much stronger. How does a woman's sexual urge compare to a man's sexual urge? Well, we actually don't know.

There is no test case where we can raise a woman outside a patriarchal society, where she's not constantly told that a woman's sex drive is much weaker than a man's sex drive. Where she's not told repeatedly that women don't really enjoy sex. That it's "normal" for a woman to fake an orgasm during sex to placate a man's ego, and that it doesn't matter if she experiences pleasure during sex.

It's true I don't know the extent of men's sexual urges. But I do know bodily urges. I know urges that all humans experience that are far, far stronger than sexual urges for both men and women—the urge to urinate and defecate. They are urges so strong that even if I ignore them, my body will take over and act on those urges.

All humans know those urges, but we have learned to control them. We don't just pee anywhere. We pee in toilets. We hold it till we get there. We build bathroom breaks into our day.

We're not asking too much when we expect men to control their sexual urges, when we expect men to take responsibility for their own bodily fluids, when we expect men to ejaculate responsibly.

**MEN CAN
EASILY
PREVENT
ABORTIONS
BUT CHOOSE
NOT TO.**

Most abortions are elective and due to unwanted pregnancy. But there are some abortions that happen during *wanted* pregnancies, and without exception they are heartbreaking. The fetus dies in utero or has medical issues that are incompatible with life or the mother has a medical issue that won't allow her body to support the pregnancy further. Again, these are heartbreaking circumstances, and thankfully they make up a very small percentage of abortions.

I point that out because men could easily prevent elective abortion, which is virtually all abortion, simply by ejaculating responsibly.

Men mostly run our government. Men mostly make the laws. For almost fifty years, a lot of men were focused on what it would take to overturn *Roe v. Wade*, claiming they wanted to reduce abortions. And then, in June 2022, the Supreme Court, made up of mostly men, *did* overturn *Roe v. Wade*.

Five decades of work. It's strange when you think about it, because if men were actually interested in reducing abortion, it didn't need to take fifty years. At any point, men could have eliminated elective abortions in a very short amount of time—a matter of weeks—without ever touching an abortion law, without legislating about women's bodies, without even mentioning women. All men had to do was ejaculate responsibly.

They chose not to.

Today, they continue to choose not to.